

prate, such as on the playground, the hyperactivity seen in children with this disorder may not be obvious.

Typically, the symptoms of this disorder in any given child vary with situation and time. A child's behavior may be well-organized and appropriate on a one-to-one basis but become dysregulated in a group situation or in the classroom; or home adjustment may be satisfactory and difficulties may emerge only in school. It is the rare child who displays signs of the disorder in all settings or even in the same setting at all times.

Associated features. Associated features vary as a function of age and include obstinacy, stubbornness, negativism, bossiness, bullying, increased mood lability, low frustration tolerance, temper outbursts, low self-esteem, and lack of response to discipline.

Specific Developmental Disorders are common, and should be noted on Axis II.

Nonlocalized "soft" neurological signs, motor-perceptual dysfunctions (e.g., poor eye-hand coordination), and EEG abnormalities may be present. However, in only about 5% of the cases is Attention Deficit Disorder associated with a diagnosable neurological disorder, which should be coded on Axis III.

Age at onset. Onset is typically by the age of three, although frequently the disorder does not come to professional attention until the child enters school.

Course. There are three characteristic courses. In the first, all of the symptoms persist into adolescence or adult life. In the second, the disorder is self-limited and all of the symptoms disappear completely at puberty. In the third, the hyperactivity disappears, but the attentional difficulties and impulsivity persist into adolescence or adult life (Residual Type). The relative frequency of these courses is unknown.

Impairment. Academic difficulties are common; and although impairment may be limited to academic functioning, social functioning may be impaired as well. Infrequently children with this disorder require residential treatment.

Complications. School failure, Conduct Disorder, and Antisocial Personality Disorder are the major complications.

Predisposing factors. Mild or Moderate Mental Retardation, epilepsy, some forms of cerebral palsy, and other neurological disorders may be predisposing factors.

Prevalence. The disorder is common. In the United States, it may occur in as many as 3% of prepubertal children.

Sex ratio. The disorder is ten times more common in boys than in girls.

Familial pattern. The disorder is apparently more common in family members than in the general population.

Differential diagnosis. **Age-appropriate overactivity**, as is seen in some particularly active children, does not have the haphazard and poorly organized quality characteristic of the behavior of children with Attention Deficit Disorder. Children in **inadequate, disorganized, or chaotic environments** may appear to have difficulty in sustaining attention and in goal-directed behavior. In such cases it may be impossible to determine whether the disorganized behavior is simply a function of the chaotic environment or whether it is due to the child's psychopathology (in which case the diagnosis of Attention Deficit Disorder may be warranted).

In **Severe and Profound Mental Retardation** there may be clinical features that are characteristic of Attention Deficit Disorder. However, the additional diagnosis of Attention Deficit Disorder would make clinical sense only if the Mental Retardation were Mild or Moderate in severity.

Many cases of **Conduct Disorder** have signs of impulsivity, inattention, and hyperactivity. The additional diagnosis of Attention Deficit Disorder is frequently warranted.

In **Schizophrenia and Affective Disorders with manic features** there may be clinical features that are characteristic of Attention Deficit Disorder. However, these diagnoses preempt the diagnosis of Attention Deficit Disorder.

Diagnostic criteria for Attention Deficit Disorder with Hyperactivity

The child displays, for his or her mental and chronological age, signs of developmentally inappropriate inattention, impulsivity, and hyperactivity. The signs must be reported by adults in the child's environment, such as parents and teachers. Because the symptoms are typically variable, they may not be observed directly by the clinician. When the reports of teachers and parents conflict, primary consideration should be given to the teacher reports because of greater familiarity with age-appropriate norms. Symptoms typically worsen in situations that require self-application, as in the classroom. Signs of the disorder may be absent when the child is in a new or a one-to-one situation.

The number of symptoms specified is for children between the ages of eight and ten, the peak age range for referral. In younger children, more severe forms of the symptoms and a greater number of symptoms are usually present. The opposite is true of older children.

A. Inattention. At least three of the following:

- (1) often fails to finish things he or she starts
- (2) often doesn't seem to listen
- (3) easily distracted
- (4) has difficulty concentrating on schoolwork or other tasks requiring sustained attention

(5) has difficulty sticking to a play activity

B. Impulsivity. At least three of the following:

- (1) often acts before thinking
- (2) shifts excessively from one activity to another
- (3) has difficulty organizing work (this not being due to cognitive impairment)
- (4) needs a lot of supervision
- (5) frequently calls out in class
- (6) has difficulty awaiting turn in games or group situations

C. Hyperactivity. At least two of the following:

- (1) runs about or climbs on things excessively
- (2) has difficulty sitting still or fidgets excessively
- (3) has difficulty staying seated
- (4) moves about excessively during sleep
- (5) is always “on the go” or acts as if “driven by a motor”

D. Onset before the age of seven.

E. Duration of at least six months.

F. Not due to Schizophrenia, Affective Disorder, or Severe or Profound Mental Retardation.

314.00 Attention Deficit Disorder without Hyperactivity

All of the features are the same as those of Attention Deficit Disorder with Hyperactivity except for the absence of hyperactivity; the associated features and impairment are generally milder. Prevalence and familial pattern are unknown.

Diagnostic criteria for Attention Deficit Disorder without Hyperactivity

The criteria for this disorder are the same as those for Attention Deficit Disorder with Hyperactivity except that the individual never had signs of hyperactivity (criterion C).

314.80 Attention Deficit Disorder, Residual Type

Diagnostic criteria for Attention Deficit Disorder, Residual Type

A. The individual once met the criteria for Attention Deficit Disorder with Hyperactivity. This information may come from the individual or from others, such as family members.

B. Signs of hyperactivity are no longer present, but other signs of the illness have persisted to the present without periods of remission, as evidenced by signs of both attentional deficits and impulsivity (e.g., difficulty organizing work and completing tasks, difficulty concentrating, being easily distracted, making sudden decisions without thought of the consequences).

C. The symptoms of inattention and impulsivity result in some impairment in social or occupational functioning.

D. Not due to Schizophrenia, Affective Disorder, Severe or Profound Mental Retardation, or Schizotypal or Borderline Personality Disorders.

CONDUCT DISORDER

The essential feature is a repetitive and persistent pattern of conduct in which either the basic rights of others or major age-appropriate societal norms or rules are violated. The conduct is more serious than the ordinary mischief and pranks of children and adolescents.

Four specific subtypes are included: Undersocialized, Aggressive; Undersocialized, Nonaggressive; Socialized, Aggressive; and Socialized, Nonaggressive. These subtypes are based on the presence or absence of adequate social bonds and the presence or absence of a pattern of aggressive antisocial behavior. The validity of these diagnostic subtypes within the category of Conduct Disorder is controversial. Some investigators believe that a more useful distinction would be on the basis of the variety, frequency, and seriousness of the antisocial behavior rather than the type of disturbance, whereas others believe that the Undersocialized and Socialized types represent distinct disorders.

The *Undersocialized* types are characterized by a failure to establish a normal degree of affection, empathy, or bond with others. Peer relationships are generally lacking, although the youngster may have superficial relationships with other youngsters. Characteristically the child does not extend himself or herself for others unless there is an obvious immediate advantage. Egocentrism is shown by readiness to manipulate others for favors without any effort to reciprocate. There is generally a lack of concern for the feelings, wishes, and well-being of others, as shown by callous behavior. Appropriate feelings of guilt or remorse are generally absent. Such a child may readily inform on his or her companions and try to place blame on them.

The *Socialized* types show evidence of social attachment to others, but may be similarly callous or manipulative toward persons to whom they are not attached and lack guilt when these "outsiders" are made to suffer.

The *Aggressive* types are characterized by a repetitive and persistent pattern of aggressive conduct in which the rights of others are violated, by either physical violence against persons, or thefts outside the home involving confrontation with a victim. The physical violence may take the form of rape, mugging, assault, or, in rare cases, homicide. In some cases, the physical violence may be

directed against parents. Thefts outside the home may involve extortion, purse-snatching, or holdup of a store.

The *Nonaggressive* types are characterized by the *absence* of physical violence against persons and of robbery outside the home involving confrontation with a victim. However, there is a persistent pattern of conduct in conflict with norms for their age, which may take the form of: chronic violations of a variety of important rules that are reasonable and age-appropriate for the child at home or at school, such as persistent truancy and substance abuse; running away from home overnight while living in the parental home; persistent serious lying in and out of the home; vandalism or fire-setting; or stealing (not involving confrontation of a victim).

Associated features (all four types). Difficulties at home and in the community are common. Frequently there is precocious sexual activity, which may be aggressive or submissive, depending on subtype. The child typically blames others for his or her difficulties and feels unfairly treated and mistrustful of others. Self-esteem is usually low, though the individual may project an image of "toughness." Unusually early smoking, drinking, and other substance use are also common. Poor frustration tolerance, irritability, temper outbursts, and provocative recklessness are often present. Academic achievement is frequently below the level expected on the basis of intelligence and age. Attentional difficulties are common, and may justify the additional diagnosis of Attention Deficit Disorder. One or more Specific Developmental Disorders may be present.

In the Socialized types, sometimes there is membership in a gang and the antisocial behavior may be limited to gang activities.

Age at onset. Onset is usually prepubertal for the Undersocialized type and pubertal or postpubertal for the Socialized type.

Course. The course is variable, mild forms frequently showing improvement over time and severe forms tending to be chronic. Some individuals may continue their antisocial behavior and generally poor social functioning into adulthood, particularly the Undersocialized, Aggressive type, and thus qualify for the diagnosis of Antisocial Personality Disorder. Others may display adequate social functioning but persist in illegal activity, and may be considered to have Adult Antisocial Behavior (V Code). Finally, many achieve reasonable social and occupational adjustment as adults, particularly the Socialized, Nonaggressive type.

Impairment. The degree of impairment varies from mild to severe. It may preclude attendance in the ordinary school classroom or living at home or in a foster home. When antisocial behavior is extreme, institutionalization, with its temporary loss of autonomy, may be necessary.

Complications. Complications include school suspension, legal difficulties, Substance Use Disorders, venereal diseases, unwanted pregnancy, high rate of

physical injury from accidents, fights, along with retaliation by victims, and suicidal behavior.

Predisposing factors. Attention Deficit Disorder, parental rejection, inconsistent management with harsh discipline, early institutional living, frequent shifting of parent figures (foster parents, relatives, or stepparents), and being an illegitimate only child may predispose to the development of the Undersocialized type. Large family size, association with a delinquent subgroup, and an absent father or a father with Alcohol Dependence may predispose to the development of the Socialized type.

Prevalence. The disorder is common, particularly the Socialized, Nonaggressive and the Undersocialized, Aggressive types.

Sex ratio. The disorder is far more common among boys than among girls, the ratios ranging from 4:1 to 12:1. The only exception may be the Undersocialized, Nonaggressive type, which may be equally common in both sexes.

Familial pattern. The disorder is more common in children of adults with Antisocial Personality Disorder and Alcohol Dependence than in the general population.

Differential diagnosis. Isolated acts of antisocial behavior do not justify a diagnosis of Conduct Disorder and may be coded as Childhood or Adolescent Antisocial Behavior (V Codes). The behavior qualifies for a diagnosis of Conduct Disorder only if the antisocial behavior represents a repetitive and persistent pattern. When such a pattern exists there will usually be obvious impairment in social and school functioning that frequently will not be present when the antisocial behavior represents an isolated act.

In **Oppositional Disorder** there are some of the features that are present in Conduct Disorder, such as disobedience and opposition to authority figures. However, the basic rights of others and major age-appropriate societal norms or rules are not violated as they are in Conduct Disorder.

Attention Deficit Disorder and **Specific Developmental Disorder** are common associated diagnoses, and should also be noted when present.

312.00 Conduct Disorder, Undersocialized, Aggressive

Diagnostic criteria

A. A repetitive and persistent pattern of aggressive conduct in which the basic rights of others are violated, as manifested by either of the following:

- (1) physical violence against persons or property (not to defend someone else or oneself), e.g., vandalism, rape, breaking and entering, fire-setting, mugging, assault

- (2) thefts outside the home involving confrontation with the victim (e.g., extortion, purse-snatching, armed robbery)

B. Failure to establish a normal degree of affection, empathy, or bond with others as evidenced by *no more than one* of the following indications of social attachment:

- (1) has one or more peer-group friendships that have lasted over six months
- (2) extends himself or herself for others even when no immediate advantage is likely
- (3) apparently feels guilt or remorse when such a reaction is appropriate (not just when caught or in difficulty)
- (4) avoids blaming or informing on companions
- (5) shares concern for the welfare of friends or companions

C. Duration of pattern of aggressive conduct of at least six months.

D. If 18 or older, does not meet the criteria for Antisocial Personality Disorder.

312.10 Conduct Disorder, Undersocialized, Nonaggressive

Diagnostic criteria

A. A repetitive and persistent pattern of nonaggressive conduct in which either the basic rights of others or major age-appropriate societal norms or rules are violated, as manifested by any of the following:

- (1) chronic violations of a variety of important rules (that are reasonable and age-appropriate for the child) at home or at school (e.g., persistent truancy, substance abuse)
- (2) repeated running away from home overnight
- (3) persistent serious lying in and out of the home
- (4) stealing not involving confrontation with a victim

B. Failure to establish a normal degree of affection, empathy, or bond with others as evidenced by *no more than one* of the following indications of social attachment:

- (1) has one or more peer-group friendships that have lasted over six months
- (2) extends himself or herself for others even when no immediate advantage is likely
- (3) apparently feels guilt or remorse when such a reaction is appropriate (not just when caught or in difficulty)
- (4) avoids blaming or informing on companions
- (5) shows concern for the welfare of friends or companions

C. Duration of pattern of nonaggressive conduct of at least six months.

D. If 18 or older, does not meet the criteria for Antisocial Personality Disorder.

312.23 Conduct Disorder, Socialized, Aggressive

Diagnostic criteria

A. A repetitive and persistent pattern of aggressive conduct in which the basic rights of others are violated, as manifested by either of the following:

- (1) physical violence against persons or property (not to defend someone else or oneself), e.g., vandalism, rape, breaking and entering, fire-setting, mugging, assault
- (2) thefts outside the home involving confrontation with a victim (e.g., extortion, purse-snatching, armed robbery)

B. Evidence of social attachment to others as indicated by at least two of the following behavior patterns:

- (1) has one or more peer-group friendships that have lasted over six months
- (2) extends himself or herself for others even when no immediate advantage is likely
- (3) apparently feels guilt or remorse when such a reaction is appropriate (not just when caught or in difficulty)
- (4) avoids blaming or informing on companions
- (5) shows concern for the welfare of friends or companions

C. Duration of pattern of aggressive conduct of at least six months.

D. If 18 or older, does not meet the criteria for Antisocial Personality Disorder.

312.21 Conduct Disorder, Socialized, Nonaggressive

Diagnostic criteria

A. A repetitive and persistent pattern of nonaggressive conduct in which either the basic rights of others or major age-appropriate societal norms or rules are violated, as manifested by any of the following:

- (1) chronic violations of a variety of important rules (that are reasonable and age-appropriate for the child) at home or at school (e.g., persistent truancy, substance abuse)
- (2) repeated running away from home overnight
- (3) persistent serious lying in and out of the home
- (4) stealing not involving confrontation with a victim

B. Evidence of social attachment to others as indicated by at least two of the following behavior patterns:

- (1) has one or more peer-group friendships that have lasted over six months
- (2) extends himself or herself for others even when no immediate advantage is likely
- (3) apparently feels guilt or remorse when such a reaction is appropriate (not just when caught or in difficulty)
- (4) avoids blaming or informing on companions
- (5) shows concern for the welfare of friends or companions

C. Duration of pattern of nonaggressive conduct of at least six months.

D. If 18 or older, does not meet the criteria for Antisocial Personality Disorder.

312.90 Atypical Conduct Disorder

This is a residual category for illnesses in which the predominant disturbance involves a pattern of conduct in which there is violation of either the basic rights of others or major age-appropriate societal norms or rules but which cannot be classified as one of the specified subtypes of Conduct Disorder.

ANXIETY DISORDERS OF CHILDHOOD OR ADOLESCENCE

This subclass includes three disorders in which anxiety is the predominant clinical feature. In the first two categories, Separation Anxiety Disorder and Avoidant Disorder of Childhood or Adolescence, the anxiety is focused on specific situations. In the third category, Overanxious Disorder, the anxiety is generalized to a variety of situations.

309.21 Separation Anxiety Disorder

The essential feature is a clinical picture in which the predominant disturbance is excessive anxiety on separation from major attachment figures or from home or other familiar surroundings. When separation occurs, the child may experience anxiety to the point of panic. The reaction is beyond that expected at the child's developmental level.

Children with Separation Anxiety Disorder are uncomfortable when they travel independently away from the house or from familiar areas. They may refuse to visit or sleep at friends' homes, to go on errands, or to attend camp or school.* They may be unable to stay in a room by themselves, and may display clinging behavior, staying close to the parent, "shadowing" the parent around the house. Physical complaints, such as stomachaches, headaches, nausea, and vomiting, are common when separation is anticipated or occurs.

* School refusal is often inaccurately referred to as "school phobia." However, not all school refusal is due to separation anxiety. When separation anxiety accounts for school refusal, the child experiences difficulty being separated from home or family for a variety of purposes, school attendance being only one of them. In a true school phobia, the child fears the school situation, whether or not he or she is accompanied by the parent.

Cardiovascular symptoms such as palpitations, dizziness, and faintness are rare in younger children, but may occur in adolescents.

When separated from significant others to whom they are attached, these children are often preoccupied with morbid fears that accidents or illness will befall their parents or themselves. They often express fear of getting lost and never being reunited with their parents. The exact nature of the fantasized mishaps varies. In general, young children have less specific, more amorphous concerns. As the child gets older, the fears may become systematized around identifiable potential dangers. Such concerns vary greatly; and many children, even some older ones, do not report fears of definite threats, but only pervasive anxiety about ill-defined dangers or death. In addition, children typically exhibit anticipatory anxiety when separation is threatened or impending. In young children, whose immature cognitive development precludes the formation of well-defined morbid worries, the mechanism of anticipatory anxiety has not yet developed, and there is distress only when separation actually occurs.

Children with this disorder often have fears of animals, monsters, and situations that are perceived as presenting danger to the integrity of the family or themselves; consequently, they may have exaggerated fears of muggers, burglars, kidnappers, car accidents, or plane travel. Concerns about dying are common.

These children often have difficulty going to sleep, and may require that someone stay with them until they fall asleep. They may make their way to their parents' bed (or that of another significant person, such as a sibling); if entry to the parental bedroom is barred, they may sleep outside the parents' door. Nightmares, whose content expresses the child's morbid fears, may occur.

Some children do not show morbid apprehension about possible harm befalling them or those close to them, but instead experience acute homesickness and feel uncomfortable to the point of misery and even panic when away from home. These children yearn to return home, and are preoccupied with reunion fantasies.

Children may refuse to see former friends or relatives to avoid accounting for their difficulties while in school or their absence from school. On occasion a child may become violent toward an individual who is forcing separation.

Adolescents with this disorder, especially boys, may deny overconcern about their mother or the wish to be with her; yet their behavior reflects anxiety about separation. Thus, they are reluctant or unable to leave the home or the parent and feel comfortable only in situations in which no separation is demanded.

Although the disorder represents a form of phobia, it is not included among the Phobic Disorders because it has unique features and is characteristically associated with childhood.

Associated features. Fear of the dark is common, and some children have fixed fears that may appear bizarre. For example, they may report that they see and feel eyes staring at them in the dark, that mythical animals are glaring at them, or that bloody creatures are reaching for them.

Children with this disorder are often described as demanding, intrusive, and in need of constant attention. Others are described as unusually conscientious, conforming, and eager to please.

When no demands for separation are made, children with Separation Anxiety Disorder typically have no interpersonal difficulties. They may complain that no one loves them or cares about them and that they wish they were dead.

Age at onset. The age at onset may be as early as preschool age. The extreme form of the disorder, involving school refusal, seems to begin most often around ages 11 and 12.

Course. Typically there are periods of exacerbation and remission over a period of several years. In some exceptional cases both the anxiety about possible separation and the avoidance of situations involving separation (e.g., going away to college) persist for many years.

Impairment. In its severe form the disorder may be very incapacitating in that the child is unable to attend school and function independently in a variety of areas.

Complications. The child often undergoes elaborate physical examinations because of numerous somatic complaints. When school refusal occurs, common complications are academic difficulties and social avoidance.

Predisposing factors. No specific premorbid personality disturbance is associated with Separation Anxiety Disorder. In most cases the disorder develops after some life stress, typically a loss, the death of a relative or pet, an illness of the child or a relative, or a change in the child's environment such as a school change or a move to a new neighborhood.

Children with this disorder tend to come from families that are close-knit and caring. The etiologic significance of this familial pattern is not clear. Neglected children are underrepresented among those with Separation Anxiety Disorders.

Prevalence. The disorder is apparently not uncommon.

Sex ratio. The disorder is apparently equally common in both sexes.

Familial pattern. The disorder is apparently more common in family members than in the general population.

Differential diagnosis. In early childhood some degree of separation anxiety is a normal phenomenon. Clinical judgment must be used to distinguish this from the clearly excessive reaction to separation seen in Separation Anxiety Disorder. In **Overanxious Disorder** and **Avoidant Disorder of Childhood** or

Adolescence anxiety is not focused on separation. In **Pervasive Developmental Disorder** or **Schizophrenia** anxiety about separation may occur, but is viewed as due to these conditions rather than as a separate disorder. In **Major Depression** occurring in children, the diagnosis Separation Anxiety Disorder should also be made when the criteria are met for both disorders, since it is difficult to know which condition should be regarded as the primary disorder. In **Conduct Disorder** truancy is common, but the child stays outside of the home and anxiety about separation is usually not present.

Diagnostic criteria for Separation Anxiety Disorder

A. Excessive anxiety concerning separation from those to whom the child is attached, as manifested by at least three of the following:

- (1) unrealistic worry about possible harm befalling major attachment figures or fear that they will leave and not return
- (2) unrealistic worry that an untoward calamitous event will separate the child from a major attachment figure, e.g., the child will be lost, kidnapped, killed, or be the victim of an accident
- (3) persistent reluctance or refusal to go to school in order to stay with major attachment figures or at home
- (4) persistent reluctance or refusal to go to sleep without being next to a major attachment figure or to go to sleep away from home
- (5) persistent avoidance of being alone in the home and emotional upset if unable to follow the major attachment figure around the home
- (6) repeated nightmares involving theme of separation
- (7) complaints of physical symptoms on school days, e.g., stomachaches, headaches, nausea, vomiting
- (8) signs of excessive distress upon separation, or when anticipating separation, from major attachment figures, e.g., temper tantrums or crying, pleading with parents not to leave (for children below the age of six, the distress must be of panic proportions)
- (9) social withdrawal, apathy, sadness, or difficulty concentrating on work or play when not with a major attachment figure

B. Duration of disturbance of at least two weeks.

C. Not due to a Pervasive Developmental Disorder, Schizophrenia, or any other psychotic disorder.

D. If 18 or older, does not meet the criteria for Agoraphobia.

313.21 Avoidant Disorder of Childhood or Adolescence

The essential feature is a clinical picture in which the predominant disturbance

is a persistent and excessive shrinking from contact with strangers of sufficient severity so as to interfere with social functioning in peer relationships, coupled with a clear desire for affection and acceptance, and relationships with family members and other familiar figures that are warm and satisfying.

Children with this disorder may cling and whisper to their caretakers, and become tearful and anxious when confronted with even trivial demands for contact with strangers. Social avoidance may be indicated by inhibition of motor action or initiative. Although there is no impairment in communicative skills, such children may seem inarticulate or even mute when social anxiety is severe. Embarrassment and timidity are conveyed by these children, although they seem interested and eager for social relationships.

Associated features. Usually such children are unassertive and lack self-confidence. In adolescence, inhibition of normal psychosexual activity may be noted.

Age at onset. The disorder may develop as early as two and a half years, after stranger anxiety as a normal developmental phenomenon should have disappeared.

Course. The course seems variable, some children improving spontaneously while others experience an episodic or chronic course. How often this disorder becomes chronic and continues into adulthood as Avoidant Personality Disorder is unknown. In general, however, it is believed that this rarely occurs.

Impairment. Age-appropriate socialization skills may not develop. It is rare for the impairment in functioning to be severe.

Predisposing factors. No information.

Complications. The most serious complication is failure to form social bonds beyond the family, with resulting feelings of isolation and depression.

Prevalence. The disorder is apparently uncommon.

Sex ratio and familial pattern. No information.

Differential diagnosis. **Socially reticent** children are slow to warm up to strangers, but after a short time can respond, and suffer no impairment in peer interaction.

In **Separation Anxiety Disorder**, the anxiety is due to separation from the home or major attachment figures rather than to contact with strangers per se. In **Overanxious Disorder**, anxiety is not limited to, or focused on, contact with strangers. In **Schizoid Disorder of Childhood or Adolescence** there is also discomfort in social situations, but there is little desire for social involvement, whereas in **Avoidant Disorder of Childhood or Adolescence** there is a clear desire for affection and acceptance. **Avoidant Personality Disorder**, rather than

Avoidant Disorder of Childhood or Adolescence, should be diagnosed only if the behavioral pattern has existed for many years and the individual is at least 18 years old. In **Adjustment Disorder with Withdrawal**, the behavioral pattern of withdrawal is clearly related to a recent psychosocial stressor.

Diagnostic criteria for Avoidant Disorder of Childhood or Adolescence

- A. Persistent and excessive shrinking from contact with strangers.
- B. Desire for affection and acceptance, and generally warm and satisfying relations with family members and other familiar figures.
- C. Avoidant behavior sufficiently severe to interfere with social functioning in peer relationships.
- D. Age at least 2½. If 18 or older, does not meet the criteria for Avoidant Personality Disorder.
- E. Duration of the disturbance of at least six months.

313.00 Overanxious Disorder

The essential feature is a clinical picture in which the predominant disturbance is excessive worrying and fearful behavior that is not focused on a specific situation or object (such as separation from a parent or entering new social interaction) and that is not due to a recent psychosocial stressor. The child worries about future events, such as examinations, the possibility of injury, or inclusion in peer group activities; or about meeting expectations, such as deadlines, keeping appointments, or performing chores. An inordinate amount of time may be spent asking about the discomforts or dangers of a variety of situations. For example, routine visits to the doctor may be anticipated with excessive concern about minor procedures.

The anxiety is typically expressed as concern with competence in a variety of areas, with the focus of concern on what others will think of his or her performance. In some cases physical concomitants of anxiety are apparent, as manifested by complaints of a lump in the throat, gastrointestinal distress, headache, shortness of breath, nausea, dizziness, or other somatic discomforts. Difficulty falling asleep is common. The child may persistently complain about "feeling nervous."

There may be preoccupation with a neighbor or adult school figure who seems "mean" or critical. As the child gets older, the preoccupations usually systematize around conventional forms of judgment such as peer, social, or athletic acceptance, school grades, and even behavior of other family members that might embarrass the child.

Associated features. Children with this disorder may seem hypermature with their "precocious" concerns. Perfectionist tendencies, with obsessional self-

doubt, may be present; there may be excessive conformity and seeking of approval. Sometimes there is an excess of motor restlessness or nervous habits such as nail biting or hair pulling.

Course. The onset may be sudden or gradual, with exacerbations associated with stress. The disorder may persist into adult life, as an Anxiety Disorder, such as Generalized Anxiety Disorder or a Social Phobia.

Age at onset. No information.

Impairment. In unusually severe cases this disorder can be incapacitating and result in inability to meet realistic demands at home and in school.

Complications. Complications may include unnecessary medical evaluation for somatic symptoms, poor school performance, and failure to engage in age-appropriate activities in which there are demands for performance, such as sports.

Predisposing factors. This disorder seems more common in eldest children, small families, upper socioeconomic groups, and families in which there is a concern about performance even when the child functions at an adequate or superior level.

Prevalence. The disorder is apparently common.

Sex ratio. The disorder is apparently more common in boys than in girls.

Familial pattern. No information.

Differential diagnosis. In **Separation Anxiety Disorder**, the anxiety is focused on situations involving separation (e.g., going to school). Children with **Attention Deficit Disorder** may appear nervous and jittery, but are not unduly concerned about the future. The two disorders, however, may coexist. In **Adjustment Disorder with Anxious Mood**, the anxiety is always clearly related to the recent occurrence of a psychosocial stressor.

Overanxious Disorder should not be diagnosed when the anxiety is due to another disorder, such as **Obsessive Compulsive Disorder**, **Major Depression**, **Schizophrenia**, or **Pervasive Developmental Disorder**.

Diagnostic criteria for Overanxious Disorder

A. The predominant disturbance is generalized and persistent anxiety or worry (not related to concerns about separation), as manifested by at least four of the following:

- (1) unrealistic worry about future events

- (2) preoccupation with the appropriateness of the individual's behavior in the past
- (3) overconcern about competence in a variety of areas, e.g., academic, athletic, social
- (4) excessive need for reassurance about a variety of worries
- (5) somatic complaints, such as headaches or stomachaches, for which no physical basis can be established
- (6) marked self-consciousness or susceptibility to embarrassment or humiliation
- (7) marked feelings of tension or inability to relax

B. The symptoms in A have persisted for at least six months.

C. If 18 or older, does not meet the criteria for Generalized Anxiety Disorder.

D. The disturbance is not due to another mental disorder, such as Separation Anxiety Disorder, Avoidant Disorder of Childhood or Adolescence, Phobic Disorder, Obsessive Compulsive Disorder, Depressive Disorder, Schizophrenia, or a Pervasive Developmental Disorder.

OTHER DISORDERS OF INFANCY, CHILDHOOD, OR ADOLESCENCE

313.89 Reactive Attachment Disorder of Infancy

The essential features of this disorder are signs of poor emotional development (lack of age-appropriate signs of social responsiveness, apathetic mood) and physical development (failure to thrive), with onset before eight months of age, because of lack of adequate caretaking. The disturbance is not due to a physical disorder, Mental Retardation, or Infantile Autism. Some severe cases of this disorder have also been called "failure to thrive" or "hospitalism."

Infants with this disorder present with poorly developed social responsiveness. By two months of age visual tracking of eyes and faces may not be established; the smile response and gaze reciprocity may be absent. At four to five months the infant may fail to participate in playful, simple games with the caretaker or observer, to attempt vocal reciprocity, to reach out when he or she is to be picked up, to reach spontaneously for the mother, or to turn his or her head toward the side from which the voice of the caretaker or observer comes. At seven to eight months the infant may not yet be crawling, establishing visual or vocal communication with the caretaker, beginning to imitate the caretaker, or displaying any of the usual more subtle facial expressions of coyness, attentiveness, etc.

The child is apathetic; staring, weak cry, poor muscle tone, weak rooting and grasping reactions to attempts to feed, and low spontaneous motility are commonly observed. Excessive sleep and a rather generalized lack of interest in the environment are frequent manifestations of the disorder.

Often such infants are noticed by a pediatrician because of their failure to thrive. Since these infants frequently do not receive well-baby care, the reason for the visit to the pediatrician may be a complicating physical illness, usually infectious, or an associated feeding problem (e.g., rumination). The head circumference is generally normal, and the failure to gain weight is disproportionately greater than the failure (if any) to gain in length.

The diagnosis of Reactive Attachment Disorder of Infancy can be made only in the presence of clear evidence of lack of adequate care. This frequently requires either a home visit, observation of the interaction between the mother or surrogate parent and the infant during both feeding and nonfeeding periods, or reports from other observers. Maternal reports may not be reliable.

It is pathognomonic of this disorder that, except in cases of extreme neglect with consequent severe physical complications (e.g., starvation, dehydration, or other intercurrent physical complications that can cause death before therapeutic measures can take hold), the clinical picture can be completely reversed by adequate care. (Such care need not be provided by a single person to be effective; it can include hospitalization, for example.) Such a therapeutic response is ultimate confirmation of the diagnosis.

Associated features. Feeding disturbances may be present, in particular, rumination, regurgitations, and vomiting. Such disturbances may be related to psychosocial deprivation and may, in turn, be a central factor in malnutrition.

Child abuse is sometimes observed, but neglect is more common.

Age at onset. The diagnosis can be made as early as in the first month of life. Onset of the disorder is always before eight months, since attachments are formed by eight months if there has been adequate caretaking. A similar clinical picture with onset past the eighth month of age should be diagnosed as Major Depression if the appropriate criteria are met.

Course, impairment, and complications. If care remains grossly inadequate, severe malnutrition, intercurrent infection, and death can occur. As noted above, however, the disorder is reversible with appropriate treatment and does not recur if affectionate care is provided, preferably from a single primary caretaker. In the absence of severe physical complications and with the proper treatment, psychological complications are likely to be limited to the effects that long-term institutionalization, if a factor in a particular case, may have on personality development.

Predisposing factors. All factors that interfere with early emotional "bonding" can predispose to this disorder.

With regard to the caretaker, these include: severe depression, maternal isolation and lack of support systems, obsessions of infanticide that make the mother stay away from the infant, maternal neglect and indifference toward the infant, extreme deprivation during the mother's own childhood, and lack of opportunities to learn about maternal behavior.

Babies that are "difficult" or very lethargic may frustrate the caretaker

excessively and enhance chances of the disorder's developing; but the role of such factors is not entirely clear.

Other factors that predispose to the disorder are lack of body-to-body contact during the first weeks of life, such as a prolonged period in an incubator or other early separations from a caring adult.

The relative contribution of psychosocial deprivation and simple food deprivation to the development of this disorder is unknown, but it is likely that both factors play a part in the majority of cases.

Prevalence, sex ratio, and familial pattern. No information.

Differential diagnosis. Children with **Mental Retardation** develop slowly, but show no medically unexplainable failure to thrive unless Reactive Attachment Disorder of Infancy is also present. Children with **Infantile Autism** may display lack of attachment behavior as infants, but they usually show no failure to thrive, and there is generally no evidence of lack of caretaking. However, Infantile Autism and Reactive Attachment Disorder of Infancy can coexist.

Children with a variety of **severe neurological abnormalities** such as **deafness, blindness, profound multisensory defects, major CNS disease, or severe chronic physical illness** may have very specific needs and few means of satisfying them and thus may suffer minor secondary attachment disturbances.

In **psychosocial dwarfism** there may also be apathy, parental neglect, and disappearance of symptoms with hospitalization. However, psychosocial dwarfism generally has a later onset than Reactive Attachment Disorder of Infancy, and the failure of the infant to gain in length with little change or actual increase in weight is the major manifestation.

Major Depression should be considered if this clinical picture develops after eight months of age.

Diagnostic criteria for Reactive Attachment Disorder of Infancy

A. Age at onset before eight months.

B. Lack of the type of care that ordinarily leads to the development of affectional bonds to others, e.g., gross emotional neglect, imposed social isolation in an institution.

C. Lack of developmentally appropriate signs of social responsivity, as indicated by at least several of the following (the total number of behaviors looked for will depend on the chronological age of the child, corrected for prematurity):

- (1) lack of visual tracking of eyes and faces by an infant more than two months of age
- (2) lack of smiling in response to faces by an infant more than two months of age

(3) lack of visual reciprocity in an infant of more than two months; lack of vocal reciprocity with caretaker in an infant of more than five months

(4) lack of alerting and turning toward caretaker's voice by an infant of more than four months

(5) lack of spontaneous reaching for the mother by an infant of more than four months

(6) lack of anticipatory reaching when approached to be picked up, by an infant more than five months of age

(7) lack of participation in playful games with caretaker by an infant of more than five months

D. At least three of the following:

(1) weak cry

(2) excessive sleep

(3) lack of interest in the environment

(4) hypomotility

(5) poor muscle tone

(6) weak rooting and grasping in response to feeding attempts

E. Weight loss or failure to gain appropriate amount of weight for age unexplainable by any physical disorder. In these cases usually the failure to gain weight (falling weight percentile) is disproportionately greater than failure to gain length; head circumference is normal.

F. Not due to a physical disorder, Mental Retardation, or Infantile Autism.

G. The diagnosis is confirmed if the clinical picture is reversed shortly after institution of adequate caretaking, which frequently includes short-term hospitalization.

313.22 Schizoid Disorder of Childhood or Adolescence

The essential feature is a defect in the capacity to form social relationships that is not due to any other mental disorder, such as Pervasive Developmental Disorder; Conduct Disorder, Undersocialized, Nonaggressive; or any psychotic disorder, such as Schizophrenia.

Children with this disorder have no close friend of similar age other than a relative or a similarly socially isolated child. They do not appear distressed by their isolation, show little desire for social involvement, and prefer to be "loners," although they may be attached to a parent or other adult. When placed in social situations, they are uncomfortable, inept, and awkward. They have no interest in activities that involve other children, such as team sports and clubs. They often appear aloof, reserved, withdrawn, and seclusive.

Associated features. These children may be belligerent and irritable, especially when demands for social performance are made. They are erratically

sensitive to criticism, displaying occasional outbursts of aggressive behavior. They are frequently scapegoated by their peers.

These children often are vague about their goals, indecisive, absentminded, and detached from their environment ("not with it" or "in a fog"). They often appear self-absorbed and engage in excessive daydreaming. They tend to pursue solitary interests and hobbies and are often preoccupied with esoteric topics, such as violence or supernatural phenomena. However, they show no loss of reality testing.

Age at onset. The disorder always begins in childhood and can be differentiated from normal social reticence as early as five years of age.

Course. The course is not well known. In some there may be increased withdrawal and detachment in adolescence and continuity with adult Schizoid Personality Disorder or Schizophrenia. In others the disorder is self-limited, with increased socialization during adolescence.

Impairment. Social relations are, by definition, severely restricted, with failure to learn the social skills necessary for adequate social functioning. Academic performance is often impaired.

Complications. Schizoid Personality Disorder and Schizophrenia may develop as complications of this disorder.

Prevalence. This disorder is apparently rare.

Sex ratio. The disorder is much more common in boys than in girls.

Predisposing factors and familial pattern. No information.

Differential diagnosis. Children with **Avoidant Disorder of Childhood or Adolescence** are interested in social participation, but are inhibited by anxiety from forming social contacts. Peer interaction, once established, is enjoyed. In **Schizophrenia** and **Pervasive Developmental Disorders** there are psychotic symptoms or marked defects in multiple areas of functioning. In **Conduct Disorder**, **Undersocialized**, **Nonaggressive** the predominant disturbance is antisocial behavior.

Diagnostic criteria for Schizoid Disorder of Childhood or Adolescence

- A. No close friend of similar age other than a relative or a similarly socially isolated child.
- B. No apparent interest in making friends.
- C. No pleasure from usual peer interactions.
- D. General avoidance of nonfamilial social contacts, especially with peers.

E. No interest in activities that involve other children (such as team sports, clubs).

F. Duration of the disturbance of at least three months.

G. Not due to Pervasive Developmental Disorder; Conduct Disorder, Undersocialized, Nonaggressive; or any psychotic disorder, such as Schizophrenia.

H. If 18 or older, does not meet the criteria for Schizoid Personality Disorder.

313.23 Elective Mutism

The essential feature is continuous refusal to speak in almost all social situations, including at school, despite ability to comprehend spoken language and to speak. These children may communicate via gestures, by nodding or shaking the head, or, in some cases, by monosyllabic or short, monotone utterances.

Children with this disorder generally have normal language skills, though some have delayed language development and abnormalities of articulation. The refusal to speak is not, however, due to a language insufficiency or another mental disorder.

Associated features. Excessive shyness, social isolation and withdrawal, clinging, school refusal, Functional Encopresis, Functional Enuresis, compulsive traits, negativism, temper tantrums, or other controlling or oppositional behavior, particularly in the home, may be observed.

Age at onset. Although onset is usually before age five, the disturbance may come to clinical attention only with entry into school.

Course. In most cases the disturbance lasts only a few weeks or months, although in a few it continues for several years.

Impairment. There may be severe impairment in social and school functioning.

Complications. School failure and teasing or scapegoating by peers are common complications.

Predisposing factors. Maternal overprotection, speech disorders, Mental Retardation, immigration, hospitalization or trauma before age three, and entering school may be predisposing factors.

Prevalence. The disorder is apparently rare: it is found in less than 1% of child-guidance, clinical, and school-social-casework referrals.

Sex ratio. The disorder is slightly more common in girls than in boys.

Familial pattern. No information.

Differential diagnosis. In **Severe or Profound Mental Retardation**, **Pervasive Developmental Disorder**, or **Developmental Language Disorder**, there may be general inability to speak. In **Major Depression**, **Avoidant Disorder of Childhood or Adolescence**, **Overanxious Disorder**, **Oppositional Disorder**, and **Social Phobia**, there may be a general refusal to speak. However, in none of these disorders is the lack of speaking the predominant disturbance.

Children in families who have emigrated to a country of a different language may refuse to speak the new language. When comprehension of the new language is adequate but the refusal to speak persists, **Elective Mutism** should be diagnosed.

Diagnostic criteria for Elective Mutism

- A. Continuous refusal to talk in almost all social situations, including at school.
- B. Ability to comprehend spoken language and to speak.
- C. Not due to another mental or physical disorder.

313.81 Oppositional Disorder

The essential feature is a pattern of disobedient, negativistic, and provocative opposition to authority figures. The diagnosis is not made if there is a pattern in which the basic rights of others or major age-appropriate societal norms or rules are violated, in which case the diagnosis of **Conduct Disorder** is made, or if the disturbance is due to another mental disorder, such as **Schizophrenia** or **Pervasive Developmental Disorder**. If the individual is 18 years or older, the disturbance does not meet the criteria for **Passive-Aggressive Personality Disorder**.

The oppositional attitude is toward family members, particularly the parents, and toward teachers. The most striking feature is the persistence of the oppositional attitude even when it is destructive to the interests and well-being of the child or adolescent. For example, if there is a rule, it is usually violated; if a suggestion is made, the individual is against it; if asked to do something, the individual refuses or becomes argumentative; if asked to refrain from an act, the child or adolescent feels obliged to carry it out. The behavior may, in fact, deprive the individual of productive activity and pleasurable relationships.

The continually confronting quality of these individuals is typical of their style and relationships. At times they may appear to be conforming, but in their conformity they still remain provocative toward those around them. Their provocation is often directed toward adults, but may well include other children.

If the individual is thwarted, temper tantrums are likely. These children or adolescents use negativism, stubbornness, dawdling, procrastination, and passive resistance to external authority.

Usually the individual does not regard himself or herself as “oppositional,” but sees the problem as arising from other people, who are making unreasonable demands. The disorder generally causes more distress to those around him or her than to the person himself or herself.

Associated features. School and family difficulties are common. There may be use of illegal substances, such as cannabis and alcohol (before the legal age).

Age at onset. The disorder may begin as early as three years, but more commonly begins in late childhood or adolescence.

Course. The course is usually chronic and lasts for several years. In some cases there may be continuity with adult Passive-Aggressive Personality Disorder.

Impairment. The disorder can interfere with all social relationships. It may cause serious academic problems if it includes refusal to learn.

Complications. School failure is a common complication.

Predisposing factors, prevalence, sex ratio, familial pattern. No information.

Differential diagnosis. **Oppositional behavior in 18-to-36-month-old children** is part of a normal developmental phase. The diagnosis of Oppositional Disorder should be considered only if severe oppositional behavior persists beyond this period.

In **Conduct Disorder** there is violation of either the basic rights of others or major age-appropriate societal norms and rules, whereas in Oppositional Disorder the disturbance is never so severe and therefore does not extend to persistent lying, violation of major rules (e.g., truancy), theft, physical aggression, or vandalism. In some instances what first appears to be Oppositional Disorder may later turn out to be an early manifestation of Conduct Disorder.

In **Schizophrenia** and **Pervasive Developmental Disorders** there may be persistent oppositional behavior. However, if the oppositional behavior is due to these disorders, the diagnosis of Oppositional Disorder is not made.

Some cases of **Attention Deficit Disorder**, **Mental Retardation**, or **chronic Organic Mental Disorders** may also meet the criteria for this disorder, in which case both diagnoses should be made.

Diagnostic criteria for Oppositional Disorder

A. Onset after 3 years of age and before age 18.

B. A pattern, for at least six months, of disobedient, negativistic, and provocative opposition to authority figures, as manifested by at least two of the following symptoms:

- (1) violations of minor rules
- (2) temper tantrums
- (3) argumentativeness
- (4) provocative behavior
- (5) stubbornness

C. No violation of the basic rights of others or of major age-appropriate societal norms or rules (as in Conduct Disorder); and the disturbance is not due to another mental disorder, such as Schizophrenia or a Pervasive Developmental Disorder.

D. If 18 or older, does not meet the criteria for Passive-Aggressive Personality Disorder.

313.82 Identity Disorder

The essential feature is severe subjective distress regarding inability to reconcile aspects of the self into a relatively coherent and acceptable sense of self. There is uncertainty about a variety of issues relating to identity, including long-term goals, career choice, friendship patterns, sexual orientation and behavior, religious identification, moral values, and group loyalties. These symptoms last at least three months and result in impairment in social or occupational (including academic) functioning. The disturbance is not due to another mental disorder, such as Affective Disorder, Schizophrenia, or Schizophreniform Disorder; and if the individual is 18 years or older, the disturbance does not meet the criteria for Borderline Personality Disorder.

The uncertainty regarding long-term goals may be expressed as inability to choose or adopt a life pattern, for example, one dedicated to material success, or service to the community, or even some combination of the two. Conflict regarding career choice may be expressed as inability to decide on a career or as inability to pursue an apparently chosen field. Conflict regarding friendship patterns may be manifested as inability to decide the kinds of people with whom to be friendly and the degree of intimacy to have. Conflict regarding values and loyalties may include concerns over religious identification, patterns of sexual behavior, and moral issues. The individual experiences these conflicts as irreconcilable aspects of his or her personality and, as a result, fails to perceive himself or herself as having a coherent identity. Frequently, the disturbance is epitomized by the individual's asking the question "Who am I?"

Associated features. Mild anxiety and depression are common and are usually related to inner preoccupation rather than external events. Self-doubt and doubt about the future are usually present, with either difficulty in making choices or impulsive experimentation. Negative or oppositional patterns are often

chosen in an attempt to establish an independent identity distinct from family or other close individuals. Such attempts may be manifested as transient experimental phases of widely divergent behavior as the individual "tries on" various roles.

Age at onset and course. The most common age at onset is late adolescence, when individuals generally become detached from their family value systems and attempt to establish independent identities. (This diagnosis appears in this section of the manual because the most common onset is in adolescence.) As value systems change, this disorder may also appear in young adulthood, or even in middle age, if an individual begins to question earlier life decisions.

Frequently there is a phase with acute onset, which either resolves over a period of time or becomes chronic. In other instances the onset is more gradual. If the disorder begins in adolescence, it usually is resolved by the mid-20s. If it becomes chronic, however, the individual may be unable to establish a career commitment or may fail to form lasting emotional attachments, with resulting frequent shifts in jobs, relationships, and career directions.

Impairment. The degree of impairment varies. Usually there is some interference in occupational and social functioning, with deterioration in friendships and family relationships.

Complications. Educational achievement and work performance below that appropriate to the individual's intellectual ability may result from this disorder.

Prevalence. No information. The disorder is apparently more common now than several decades ago, however, perhaps because today there are more options regarding values, behavior, and life-styles open to the individual and more conflict between adolescent peer values and parental or societal values.

Predisposing factors, sex ratio, and familial pattern. No information.

Differential diagnosis. Normal conflicts associated with maturing, such as "adolescent turmoil" or "middle-age crisis," are usually not associated with severe distress and impairment in occupational or social functioning. Nevertheless, if the criteria are met, the diagnosis of Identity Disorder should be given regardless of the developmental stage of the individual.

In **Schizophrenia**, **Schizophreniform Disorder**, and **Affective Disorder** there frequently are marked disturbances in identity, but these diagnoses preempt the diagnosis of Identity Disorder.

In **Borderline Personality Disorder** identity disturbances are only one of several important areas of disturbance, and there is often considerable mood disturbance. If the individual is 18 or over and meets the criteria for Borderline Personality Disorder, then that diagnosis preempts the diagnosis of Identity Disorder. What appears initially to be Identity Disorder may later turn out to have been an early manifestation of one of the disorders noted above.

Diagnostic criteria for Identity Disorder

A. Severe subjective distress regarding uncertainty about a variety of issues relating to identity, including three or more of the following:

- (1) long-term goals
- (2) career choice
- (3) friendship patterns
- (4) sexual orientation and behavior
- (5) religious identification
- (6) moral value systems
- (7) group loyalties

B. Impairment in social or occupational (including academic) functioning as a result of the symptoms in A.

C. Duration of the disturbance of at least three months.

D. Not due to another mental disorder, such as Affective Disorder, Schizophrenia, or Schizophreniform Disorder.

E. If 18 or older, does not meet the criteria for Borderline Personality Disorder.

EATING DISORDERS

This subclass of disorders is characterized by gross disturbances in eating behavior; it includes Anorexia Nervosa, Bulimia, Pica, Rumination Disorder of Infancy, and Atypical Eating Disorder. Bulimia usually has a chronic, remitting course, whereas the other three specific disorders commonly are limited to a single episode. Two of these, Anorexia Nervosa and Rumination Disorder of Infancy, may have an unremitting course that progresses to death.

Simple obesity is included in ICD-9-CM as a physical disorder and is not in this section since it is not generally associated with any distinct psychological or behavioral syndrome. However, when there is evidence that psychological factors are of importance in the etiology or course of a particular case of obesity, this can be indicated by noting Psychological Factors Affecting Physical Condition (p. 303).

307.10 Anorexia Nervosa

The essential features are intense fear of becoming obese, disturbance of body image, significant weight loss, refusal to maintain a minimal normal body weight, and amenorrhea (in females). The disturbance cannot be accounted for by a known physical disorder. (The term "anorexia" is a misnomer, since loss of appetite is usually rare until late in the illness.)

Individuals with this disorder say they "feel fat" when they are of normal weight or even emaciated. They are preoccupied with their body size and often

gaze at themselves in a mirror. At least 25% of their original body weight is lost, and a minimal normal weight for age and height is not maintained.

The weight loss is usually accomplished by a reduction in total food intake, with a disproportionate decrease in high carbohydrate- and fat-containing foods, self-induced vomiting, use of laxatives or diuretics, and extensive exercising.

The individual usually comes to medical attention when weight loss becomes significant. When it becomes profound, physical signs such as hypothermia, dependent edema, bradycardia, hypotension, lanugo (neonatal-like hair), and a variety of metabolic changes occur. Amenorrhea often appears before noticeable weight loss has occurred.

Associated features. Some individuals with this disorder cannot exert continuous control over their intended voluntary restriction of food intake and have bulimic episodes (eating binges), often followed by vomiting. Other peculiar behavior concerning food is common. For example, individuals with this disorder often prepare elaborate meals for others, but tend to limit themselves to a narrow selection of low-calorie foods. In addition, food may be hoarded, concealed, crumbled, or thrown away.

Most individuals with this disorder steadfastly deny the illness and are uninterested in, even resistant to, therapy. Many of the adolescents have delayed psychosexual development, and adults have a markedly decreased interest in sex. Compulsive behavior, such as hand-washing, may be present during the illness. A higher than expected frequency of urogenital abnormalities and Turner's syndrome has been found in individuals with Anorexia Nervosa.

Age at onset. Age at onset is usually early to late adolescence, although it can range from prepuberty to the early 30s (rare).

Sex ratio and prevalence. This disorder occurs predominantly in females (95%). As many as 1 in 250 females between 12 and 18 years (high-risk age group) may develop the disorder.

Course. The course may be unremitting until death by starvation, episodic, or, most commonly, a single episode with full recovery.

Impairment. The severe weight loss often necessitates hospitalization to prevent death by starvation.

Complications. Follow-up studies indicate mortality rates between 15% and 21%.

Familial pattern. The disorder is more common among sisters and mothers of individuals with the disorder than in the general population.

Predisposing factors. In some individuals the onset of illness is associated with a stressful life situation. Many of these individuals are described as having

been overly perfectionist "model children." About one-third of the individuals are mildly overweight before the onset of the illness.

Differential diagnosis. In **Depressive Disorders**, and certain **physical disorders**, weight loss can occur, but there is no intense fear of obesity or disturbance of body image.

In **Schizophrenia** there may be bizarre eating patterns; however, the full syndrome of Anorexia Nervosa is rarely present; when it is, both diagnoses should be given.

In **Bulimia**, weight loss, if it does occur, is never as great as 25% of original body weight. In rare instances an episode of Anorexia Nervosa occurs in an individual with Bulimia, in which case both diagnoses are given.

Diagnostic criteria for Anorexia Nervosa

A. Intense fear of becoming obese, which does not diminish as weight loss progresses.

B. Disturbance of body image, e.g., claiming to "feel fat" even when emaciated.

C. Weight loss of at least 25% of original body weight or, if under 18 years of age, weight loss from original body weight plus projected weight gain expected from growth charts may be combined to make the 25%.

D. Refusal to maintain body weight over a minimal normal weight for age and height.

E. No known physical illness that would account for the weight loss.

307.51 Bulimia

The essential features are episodic binge eating accompanied by an awareness that the eating pattern is abnormal, fear of not being able to stop eating voluntarily, and depressed mood and self-deprecating thoughts following the eating binges. The bulimic episodes are not due to Anorexia Nervosa or any known physical disorder.

Eating binges may be planned. The food consumed during a binge often has a high caloric content, a sweet taste, and a texture that facilitates rapid eating. The food is usually eaten as inconspicuously as possible, or secretly. The food is usually gobbled down quite rapidly, with little chewing. Once eating has begun, additional food may be sought to continue the binge, and often there is a feeling of loss of control or inability to stop eating. A binge is usually terminated by abdominal pain, sleep, social interruption, or induced vomiting. Vomiting decreases the physical pain of abdominal distention, allowing either continued eating or termination of the binge, and often reduces post-binge anguish.

Although eating binges may be pleasurable, disparaging self-criticism and a depressed mood follow.

Individuals with Bulimia usually exhibit great concern about their weight and make repeated attempts to control it by dieting, vomiting, or the use of cathartics or diuretics. Frequent weight fluctuations due to alternating binges and fasts are common. Often these individuals feel that their life is dominated by conflicts about eating.

Associated features. Although most individuals with Bulimia are within a normal weight range, some may be slightly underweight and others may be overweight. Some individuals are subject to intermittent Substance Abuse, most frequently of barbiturates, amphetamines, or alcohol. Individuals may manifest undue concern with body image and appearance, often related to sexual attractiveness, with a focus on how others will see and react to them.

Age at onset. The disorder usually begins in adolescence or early adult life.

Sex ratio. The disorder occurs predominantly in females.

Course. The usual course is chronic and intermittent over a period of many years. Usually the binges alternate with periods of normal eating, or with periods of normal eating and fasts. In extreme cases, however, there may be alternate binges and fasts with no periods of normal eating.

Familial pattern. No information, although frequently obesity is present in parents or siblings.

Impairment and complications. Bulimia is seldom incapacitating except in a few individuals who spend their entire day in binge eating and self-induced vomiting. Electrolyte imbalance and dehydration can occur in those below normal weight who vomit after binges.

Prevalence and predisposing factors. No information.

Differential diagnosis. In **Anorexia Nervosa** there is severe weight loss, but in Bulimia the weight fluctuations are never so extreme as to be life-threatening. In **Schizophrenia** there may be unusual eating behavior, but the full syndrome of Bulimia is rarely present; when it is, both diagnoses should be given. In certain neurological diseases, such as **epileptic equivalent seizures**, **CNS tumors**, **Klüver-Bucy-like syndromes**, and **Klein-Levin syndrome**, there are abnormal eating patterns, but the diagnosis Bulimia is rarely warranted; when it is, both diagnoses should be given.

Diagnostic criteria for Bulimia

A. Recurrent episodes of binge eating (rapid consumption of a large amount of food in a discrete period of time, usually less than two hours).

B. At least three of the following:

- (1) consumption of high-caloric, easily ingested food during a binge
- (2) inconspicuous eating during a binge
- (3) termination of such eating episodes by abdominal pain, sleep, social interruption, or self-induced vomiting
- (4) repeated attempts to lose weight by severely restrictive diets, self-induced vomiting, or use of cathartics or diuretics
- (5) frequent weight fluctuations greater than ten pounds due to alternating binges and fasts

C. Awareness that the eating pattern is abnormal and fear of not being able to stop eating voluntarily.

D. Depressed mood and self-deprecating thoughts following eating binges.

E. The bulimic episodes are not due to Anorexia Nervosa or any known physical disorder.

307.52 Pica

The essential feature is the persistent eating of a nonnutritive substance. Infants with the disorder typically eat paint, plaster, string, hair, or cloth. Older children may eat animal droppings, sand, bugs, leaves, or pebbles. There is no aversion to food.

Associated features. There are no regularly associated features.

Age at onset. Age at onset is usually from 12 to 24 months, but may be earlier.

Course. Pica usually remits in early childhood, but may persist into adolescence or, rarely, continue through adulthood.

Impairment. None.

Complications. Lead poisoning may result from the ingestion of paint or paint-soaked plaster; hairball tumors may cause intestinal obstruction.

Predisposing factors. Mental Retardation, mineral deficiency (e.g., of zinc or iron), neglect, and poor supervision may be predisposing factors.

Prevalence and sex ratio. Pica is a rare disorder, and is reported equally for both sexes.

Familial pattern. No information.

Differential diagnosis. In **Infantile Autism**, **Schizophrenia**, and **certain physical disorders**, such as **Klein-Levin syndrome**, nonnutritive substances may be eaten. In such instances Pica should not be noted as an additional diagnosis.

Diagnostic criteria for Pica

A. Repeated eating of a nonnutritive substance for at least one month.

B. Not due to another mental disorder, such as Infantile Autism or Schizophrenia, or a physical disorder, such as Klein-Levin Syndrome.

307.53 Rumination Disorder of Infancy

The essential feature is repeated regurgitation of food, with weight loss or failure to gain expected weight, developing after a period of normal functioning. Partially digested food is brought up into the mouth without nausea, retching, disgust, or associated gastrointestinal disorder. The food is then ejected from the mouth or chewed and reswallowed. A characteristic position of straining and arching the back with the head held back is observed. Sucking movements of the tongue occur, and the infant gives the impression of gaining considerable satisfaction from the activity.

Associated features. Commonly there are irritability and hunger between episodes of regurgitation.

Age at onset. The disorder usually starts between 3 and 12 months of age. In children with Mental Retardation, it occasionally begins later.

Course. The disorder is potentially fatal. A mortality rate from malnutrition as high as 25% has been reported. In severe cases, although the infant is apparently hungry and ingests large amounts of food, progressive malnutrition occurs because regurgitation immediately follows the feedings. Spontaneous remissions are thought to be common.

Impairment. If failure to gain expected weight or severe malnutrition develops, developmental delays in all spheres often occur, and impairment can be severe.

Complications. A frequent complication of this disorder is that the caretaker becomes discouraged by failure to feed the infant successfully, and becomes alienated from the child. The noxious odor of the regurgitated material also leads to avoidance of the infant by the caretaker, with resulting understimulation.

Predisposing factors and familial pattern. No information.

Prevalence. The disorder is apparently very rare.

Sex ratio. The disorder is equally common in boys and in girls.

Differential diagnosis. Congenital anomalies, such as pyloric stenosis, or infections of the gastrointestinal system, can cause regurgitation of food and need to be ruled out by appropriate physical examination.

Diagnostic criteria for Rumination Disorder of Infancy

- A. Repeated regurgitation without nausea or associated gastrointestinal illness for at least one month following a period of normal functioning.
- B. Weight loss or failure to make expected weight gain.

307.50 Atypical Eating Disorder

This category is a residual category for eating disorders that cannot be adequately classified in any of the previous categories.

STEREOTYPED MOVEMENT DISORDERS

The essential feature of disorders in this subclass is an abnormality of gross motor movement. The specific Stereotyped Movement Disorders all involve tics and include Transient Tic Disorder, Chronic Motor Tic Disorder, and Tourette's Disorder. It is unknown whether the three tic disorders represent distinct conditions or a continuum of severity.

Differential diagnosis of tics. A tic is defined as an involuntary rapid movement of a functionally related group of skeletal muscles or the involuntary production of noises or words. Tics should be distinguished from other movement disturbances. *Choreiform movements* are dancing, random, irregular, nonrepetitive movements. *Dystonic movements* are slower, twisting movements interspersed with prolonged states of muscular tension. *Athetoid movements* are slow, irregular, writhing movements, most frequently in the fingers and toes. *Myoclonic movements* are brief, shocklike muscle contractions that may affect parts of or a whole muscle, but not entire muscle groups. *Hemiballismic movements* are intermittent, coarse, jumping, and unilateral movements of the limbs. *Spasms* are stereotypic, slower, and more prolonged than tics, and involve groups of muscles. *Hemifacial spasm* consists of irregular, repetitive, unilateral jerks of facial muscles. *Synkinesis* consists of movements of the corner of the mouth when the individual intends to close the eye, and its converse. *Dyskinesias*, such as tardive dyskinesia, are silent, oral-buccal-lingual, masticatory movements in the face and choreoathetoid movements in the limbs. The term "stereotyped movement," which is here used as a diagnostic term, refers to a voluntary, brief or prolonged habit or mannerism that often is experienced as pleasurable. (See Atypical Stereotyped Movement Disorder, p. 77.)

Associated features of Tic Disorders. Shame and self-consciousness are common, especially with Tourette's Disorder.

Impairment in Tic Disorders. Social and occupational functioning may be impaired owing to social ostracism or the interference in normal functioning caused by the tics.

307.21 Transient Tic Disorder

The essential features are recurrent involuntary, repetitive, rapid movements (tics). The movements can be voluntarily suppressed for minutes to hours. The intensity of the symptoms varies over weeks or months. The onset is during childhood or adolescence. The duration is at least one month, but not more than one year.

The most common tic is an eye blink or another facial tic. However, the whole head, torso, or limbs may be involved. In addition, there may be vocal tics. An individual may have only one or a number of tics; if the latter, the tics may be performed simultaneously, sequentially, or randomly.

Tics are exacerbated by stress or anticipation. They disappear during sleep, and may become attenuated during some absorbing activities. The individual may be successful in temporarily suppressing the tics; eventually, however, the tics reappear. From week to week or month to month, the strength and frequency of the tics will change.

Age at onset. The age at onset is always during childhood or early adolescence, and may be as early as two years of age.

Course. The tics may disappear permanently, or recur, especially during periods of stress. In rare cases, after a period of partial remission, the individual may develop Tourette's Disorder.

Complications and predisposing factors. No information.

Prevalence. Surveys of schoolchildren have reported that from 12% to 24% of the children have had a history of some kind of tic. However, since these surveys do not specify a minimum or a maximum duration, it is not known how applicable these findings are to the prevalence of Tic Disorders as defined in this manual.

Sex ratio. Most studies find the disorder three times as common in boys as in girls.

Familial pattern. Tics are apparently more common in family members than in the general population.

Differential diagnosis. In **Tourette's Disorder** and **Chronic Motor Tic Disorder** the duration of the disturbance is at least one year. In **Tourette's Disorder** there are vocal tics which are rarely present in **Transient Tic Disorder**. See differential diagnosis of tics (p. 73) and **Tourette's Disorder** (p. 77).